

Patient ID: S (Bed)Period: March 15 to April 14, 2024Report Date: April 15, 2024Coverage: 160/744h (22%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

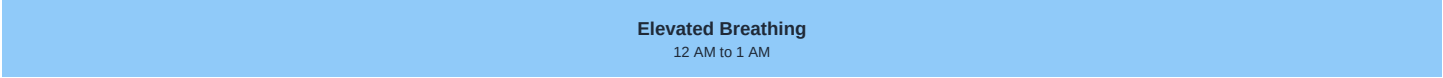
Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Last 24 Hours



Vital Stat	Avg	Min	Max
Heart Rate	66 bpm	52 bpm	79 bpm
Breathing	19 brpm	12 brpm	31 brpm

Limited data in the final 24h (12/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
12 AM to 1 AM	1h	1	Elevated Breathing	29	25	31	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 31 days from Mar 15 to Apr 14

Elevated Breathing

Apr 10 to Apr 14 (5d)

HR Breathing

Low HR

Mar 29 to Mar 30 (2d)

5 episodic events Detected, spanning 6h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Skip

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
5	6h	Low Heart Rate	<1	22%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 29	9 PM to 11 PM	3h	2	Low Heart Rate	43	40	69	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 10	8 AM to 9 AM	3h	3	Elevated Breathing	26	10	34	Check SpO2 and lung sounds; assess for fluid overload or early infection

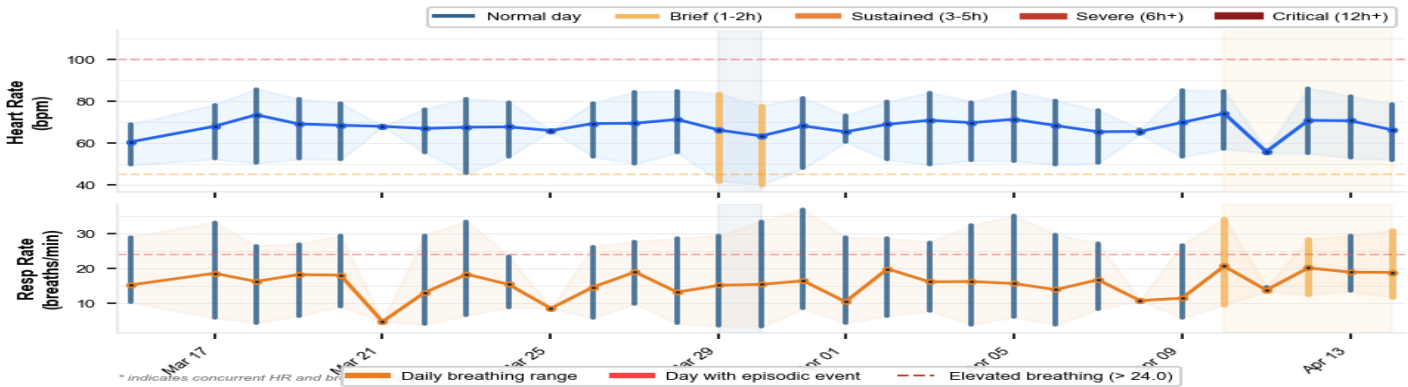
Insufficient data coverage (22%, below 30%).

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 34 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 2 unstable phases with 19 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

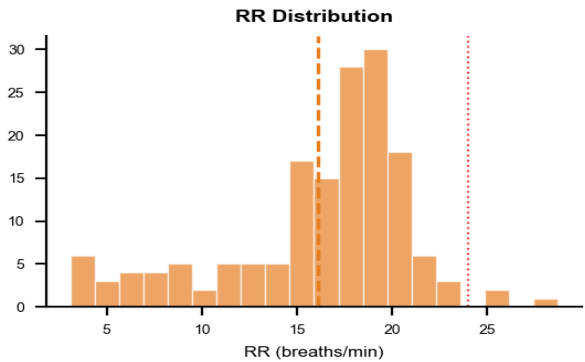
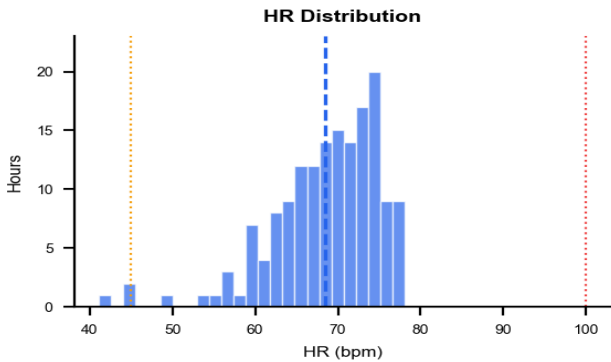
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 5 of 31 days (16%).
- 2. **Nocturnal pattern:** 4 of 5 eligible episodes during evening or night hours.

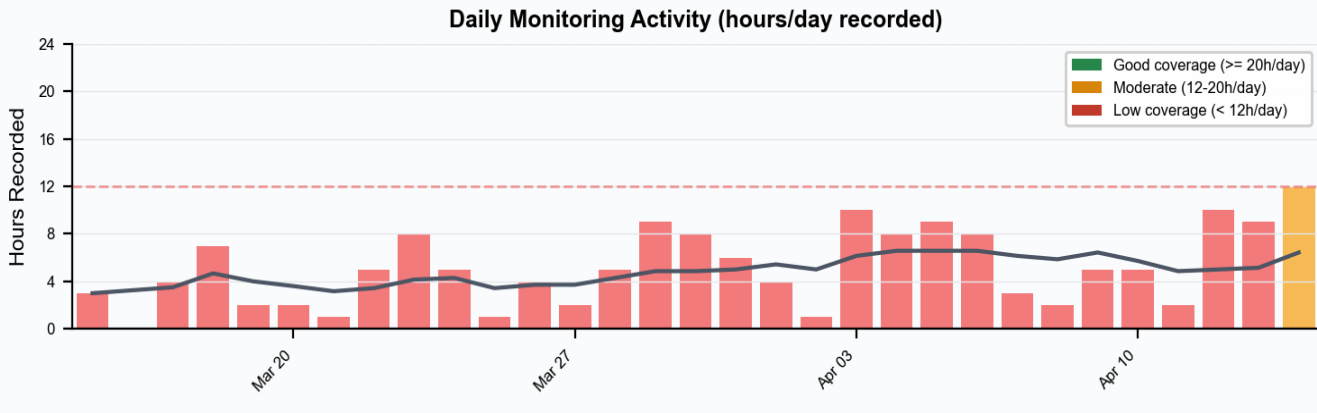


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.